

Counselor Scope of Practice

Document ID	TG-POL-02	Effective	March 1, 2026
Applies to	Counselors (LPC-Associate, Pastoral & Relationship Counselors)	Owner	Clinical & Compliance Team

At a Glance

In scope	General communication difficulties, life transition support, premarital guidance, mild situational stress, relationship psychoeducation.
Out of scope	Diagnosable trauma disorders, active management of diagnosed mental illness, eating disorders, substance use treatment, complicated/traumatic grief.
Immediate escalation	Any suicidal or homicidal ideation — see Emergency Protocol (TG-POL-05), regardless of severity or how it's phrased.
When unsure	Consult your clinical supervisor before the next session touching that topic — treat as out of scope until confirmed otherwise.

1. Purpose & Applicability

This policy defines the scope of practice for Counselors on the Togethr platform — a role positioned between Coaches and licensed Therapists. Counselors typically hold an associate-level counseling license (e.g., LPC-Associate under supervision), a pastoral counseling credential, or a master's degree in a counseling-adjacent field without independent clinical licensure.

Because Counselors do not hold an independent clinical license, their scope is intentionally narrower than a Therapist's. Where this document and the Therapist Scope of Practice (TG-POL-01) appear to overlap, the more restrictive standard applies to Counselors.

2. Credentialing Requirements

- Associate-level counselors must have an active supervision agreement on file with a Togethr-approved clinical supervisor.
- Pastoral counselors must hold a recognized pastoral counseling certification (e.g., board-certified chaplain or equivalent) and complete Togethr's relationship-counseling onboarding curriculum.
- All Counselors are re-credentialed annually.

3. Conditions and Issues Within Scope

Counselors may address:

- General relationship communication difficulties
- Life transition support (new relationship, moving in together, engagement)
- Premarital guidance, including values and expectations conversations
- Mild, situational stress or worry that does not meet diagnostic criteria for an anxiety disorder
- Psychoeducation about healthy relationship patterns, communication frameworks, and conflict de-escalation

4. Conditions Outside Counselor Scope

The following require referral to a Therapist (TG-POL-01) or a specialist per TG-POL-04, and Counselors should not attempt ongoing treatment:

- Diagnosable trauma disorders (e.g., PTSD) beyond general psychoeducation
- Active management of a diagnosed mental illness (major depressive disorder, bipolar disorder, anxiety disorders)
- Eating disorders or disordered eating patterns of any severity
- Substance use concerns beyond a single supportive conversation encouraging further evaluation
- Any grief that is complicated by trauma or accompanied by safety concerns

As with Therapists, any disclosure of suicidal or homicidal ideation triggers Emergency Protocol (TG-POL-05) immediately and supersedes all of the above.

5. Session Structure

Individual sessions are 45 minutes; couples sessions are 60 minutes. Counselors do not conduct sessions longer than the platform's standard durations without prior supervisor approval.

6. Documentation Requirements

Session summaries are completed within 24 hours using the Counselor note template, a lighter-weight version of the clinical note used by Therapists. Any content that appears to exceed Counselor scope must be flagged for supervisor review, and a referral conversation should be initiated with the client at the next available opportunity.

Notes should capture the topic discussed, any psychoeducation provided, and whether the session touched on any content bordering on out-of-scope territory, even if a referral was not yet made.

7. Escalation

Counselors escalate scope questions to their assigned clinical supervisor before the next session with the client whenever uncertainty exists. When in doubt, treat the situation as out of scope until confirmed otherwise — it is always acceptable to pause and consult.

8. Example Scenarios

- A newly engaged couple wants help discussing financial expectations before marriage. This is squarely in scope as premarital guidance.
- A client mentions feeling "really anxious lately" about a work presentation but has no history of an anxiety disorder and the concern is situational. This is in scope as mild, situational stress.
- A client describes persistent, daily panic attacks that have been going on for months and are interfering with daily functioning. This exceeds Counselor scope and should be referred to a Therapist per TG-POL-04.
- A client discloses a difficult relationship with alcohol that started after a recent loss. Even a single supportive check-in is appropriate, but ongoing treatment of the substance use itself must be referred out.

9. Frequently Asked Questions

Q: Can a Counselor provide psychoeducation about anxiety without treating an anxiety disorder?

A: Yes — general education about stress and coping strategies is in scope. Ongoing treatment of a diagnosed anxiety disorder is not.

Q: What's the difference between a Counselor and a Coach?

A: Counselors typically hold some counseling credential or supervised license and can address mild clinical content with psychoeducation; Coaches are non-clinical and must refer out any content that touches on mental health symptoms at all. See TG-POL-03 for the Coach scope.

Q: Does a Counselor need to refer immediately if a topic feels borderline?

A: Not necessarily immediately, but the uncertainty itself should be raised with a supervisor before the next session touching that topic.

10. Related Policies

This document should be read alongside: Therapist Scope of Practice (TG-POL-01), Referral Protocol (TG-POL-04), and Emergency Protocol (TG-POL-05).